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BACKGROUND GUIDE



UNODC

**Agenda: Deliberation on the combatting of
Trafficking in Persons for the Purpose of
Organ Removal (TIP for OR)**



Letter from the Executive Board

Dear Delegates,

As The Executive Board, we welcome you to this critical deliberation on one of the most pressing humanitarian crises of our time: trafficking in persons for the purpose of organ removal. This issue sits at the intersection of human rights violations, medical ethics, criminal justice, and global inequality, making it one of the most complex and urgent matters facing the international community today.

Every year, thousands of individuals fall victim to organ trafficking schemes, a practice that exploits the vulnerable, violates human dignity, and perpetuates a shadow economy that operates beyond the reach of law enforcement and regulatory oversight. The United Nations estimates that between 5-10% of global organ transplants involve organs obtained through trafficking, coercion, or other unethical means. This translates to approximately 10,000 to 20,000 organs annually being trafficked worldwide, affecting millions of vulnerable people and representing an estimated market value in the billions of dollars.

The victims of organ trafficking are invariably among the world's most vulnerable populations: individuals living in extreme poverty, migrant workers, prisoners, refugees, and other marginalized groups who have limited access to justice and protection. Meanwhile, traffickers exploit systemic gaps in regulation, corruption within medical establishments, and the desperate need for organs among patients with end-stage organ disease to create networks that operate with relative impunity.

This committee has been convened to develop comprehensive strategies to combat this grave violation of human rights and fundamental human dignity. You will be expected to examine the existing international legal frameworks, assess the effectiveness of current measures at both national and international levels, and propose innovative solutions that address the root causes of this phenomenon while simultaneously protecting vulnerable populations and holding perpetrators accountable.

We expect delegates to engage in rigorous, principled deliberation that reflects the shared commitment of the international community to human dignity, justice, and the rule of law. Your work will contribute significantly to strengthening the global response to organ trafficking and protecting some of the world's most vulnerable populations. The resolutions and commitments you develop over these three days will inform national legislation, shape bilateral and multilateral cooperation, and send a powerful message that the international community will not tolerate the commodification of human organs or the exploitation of the desperate and vulnerable.



We look forward to your constructive participation, creative problem-solving, and groundbreaking solutions that can truly make a difference in the lives of millions of people at risk of trafficking.

Respectfully,

The Executive Board

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ABOUT THE COMMITTEE

The **United Nations Office on Drugs and Crime (UNODC)** is a specialised agency within the United Nations that focuses on **addressing issues related to drugs, crime, and terrorism**. Formed in 1997 by combining the United Nations Drug Control Programme and the Centre for International Crime Prevention, UNODC has a crucial role in promoting international cooperation and implementing effective strategies to tackle these challenges. Headquartered in **Vienna**, UNODC operates through numerous field offices globally, partnering with member states to develop programs aimed at enhancing justice, security, and health. Its initiatives are based on international treaties and conventions, including the United Nations Convention against Corruption and the United Nations Convention against Transnational Organized Crime.

This committee has been established under the framework of the United Nations Office on Drugs and Crime (UNODC) and operates in close conjunction with the **World Health Organization (WHO)** and the **Office of the High Commissioner for Human Rights (OHCHR)**.

The committee brings together member states from across all regions of the world, along with international organizations, civil society representatives, and experts in medical ethics, human rights, law enforcement, and transplantation medicine. This multi-stakeholder approach ensures that the committee's work addresses organ trafficking from multiple perspectives and incorporates diverse expertise and viewpoints.

The inclusion of representatives from developed and developing nations is particularly important, as organ trafficking patterns differ significantly across regions, and solutions must be culturally appropriate and contextually relevant. Civil society organizations bring grassroots perspectives and experiences of working directly with trafficking survivors, while medical and ethics experts ensure that any proposed solutions maintain alignment with fundamental principles of medical ethics and human rights.



Mandate

The UNODC is tasked with multiple critical functions that collectively aim to create a comprehensive international response to organ trafficking.

First and foremost, the committee must assess the scale and scope of organ trafficking globally while also identifying significant regional variations. Different parts of the world face different trafficking patterns, vulnerabilities, and challenges, and any effective international response must account for these variations.

Second, the committee must rigorously evaluate the effectiveness of existing international legal instruments and national legislation that purport to address organ trafficking. Many countries have enacted laws against organ trafficking, yet prosecution remains rare and trafficking continues. Understanding why current legal frameworks are insufficient is essential for developing stronger mechanisms. The committee must also address critical gaps in enforcement and compliance mechanisms that allow traffickers to operate with relative impunity. In many cases, laws exist on paper but lack the institutional capacity, political will, or resources for effective implementation.

Third, the committee is responsible for developing coordinated strategies to both prevent trafficking before it occurs and to prosecute those who perpetrate these crimes. This requires coordination between law enforcement agencies, medical regulators, and judicial systems across multiple countries. Fourth, the committee must ensure the development of robust victim protection and rehabilitation frameworks that address the medical, psychological, social, and economic needs of trafficking survivors. For too long, victim protection has been overlooked in favor of criminal prosecution, yet survivors often face lifelong health complications and psychological trauma.

Finally, the committee is tasked with promoting transparency in organ procurement and transplantation systems worldwide. Transparency serves multiple purposes: it allows for detection of anomalies that might indicate trafficking, it builds public trust in transplantation systems, and it discourages participation by unethical actors. The committee must work toward establishing international standards for transparency that can be implemented across diverse healthcare systems.



Collaborative Efforts and Global Impact

UNODC collaborates with governments, international bodies, civil society, and the private sector to develop and implement policies that address the root causes and effects of crime and drug-related problems. It provides technical assistance, conducts research, and promotes international standards and best practices. UNODC's comprehensive approach includes supporting law enforcement, enhancing judicial and prosecutorial capabilities, and promoting community-based interventions. It also highlights the importance of public health strategies in addressing drug dependence and advocates for alternative development programs in regions affected by illegal crop cultivation.



INTRODUCTION

Human trafficking for organ removal is surrounded by myths and misconceptions. In movies, it's often portrayed as a person waking up in a bathtub filled with ice, missing a kidney. However, the reality of this crime is far more complex, and these myths hinder our comprehension of how traffickers operate.

1. What is human trafficking for organ removal?

It is a form of trafficking in which individuals are exploited for organs. Although victims often appear to have consented to the removal of their organs, their consent is invalid when deception, fraud or abuse of a position of vulnerability is involved. In such cases, they are considered victims of human trafficking.

The traffickers, who are usually part of sophisticated criminal networks, profit by selling these organs to recipients who are unable or unwilling to wait for legal transplants.

2. How prevalent is this crime globally?

In 2007, the World Health Organization (WHO) estimated that 5-10 per cent of all transplants worldwide used organs from the black market. However, with a growing and aging global population, the globalization of unhealthy lifestyles, and increased mobility, the actual number could be significantly higher.

The exact scale of this criminal activity remains unknown. Few studies have been conducted, as the crime's clandestine nature makes data collection and verification difficult. Victims may also be hesitant to step forward, as organ selling constitutes a crime in most countries.

3. Why is there a demand for unethical organ transplants?

The demand for human trafficking for organ removal largely arises from the global shortage of organs available for ethical transplant procedures. While more than 150,000 transplants are performed annually worldwide, this meets less than 10 per cent of the global demand. Desperation drives patients with organ failure to resort to obtaining organs through illegal means. The organ trade, which includes human trafficking for organ removal, is a lucrative criminal business, amounting to between 840 million to USD 1.7 billion USD annually.



4. Which organs are harvested in this form of human trafficking?

The most harvested organs from victims of trafficking in persons are kidneys, followed by parts of livers.

5. How do these traffickers operate?

Traffickers typically operate within complex and elusive global networks, requiring a sophisticated infrastructure involving medical specialists, logistical coordination and access to healthcare facilities. They connect with their victims using local advertisements, social media or via direct approaches by recruiters, who may be former victims themselves or trusted individuals within the victim's community.

These criminal networks are highly organized and flexible, often functioning as mobile units or specialized groups. Key players include brokers who coordinate logistics, recruit medical professionals, and prepare fraudulent documents. To ensure smooth operations, they rely on a wide range of facilitators such as healthcare officials, hospital administrators, customs officers and local recruiters.

Detecting this crime can be challenging, as traffickers often train victims to pretend that they are related to the recipient to evade suspicion during evaluations at hospitals or clinics.

6. Who are the victims?

Victims of this form of trafficking are typically from poor, uneducated and vulnerable backgrounds. Organized criminal groups specifically target unemployed individuals, migrants, asylum seekers and refugees. Many are coerced, deceived or see organ selling as a last resort to improve their dire situations.

Although some victims receive limited financial compensation, many receive no money at all and sometimes no post-operative care. Most of the victims are men, with two-thirds of reported cases involving male donors.

7. How do illicit organ transplants affect victims?

The long-term health consequences for victims can be devastating, with many experiencing a sharp decline in their physical condition post-surgery, along with stigmatization and depression. The psychological impacts often lead to a further deterioration in their standard of living, trapping them in a cycle of poverty and poor health.



8. What is UNODC doing to address this issue?

The UN Office on Drugs and Crime (UNODC) provides technical and legislative assistance to bolster criminal justice responses to human trafficking for organ removal. To support criminal justice practitioners, UNODC has published the Toolkit on the Investigation and Prosecution of Trafficking in Persons for Organ Removal.

UNODC is developing research and capacity-building programmes, along with guidance, to enhance national capabilities in addressing human trafficking for organ removal and organ trafficking and is ready to provide capacity-strengthening to interested States.



HISTORICAL CONTEXT

1. The Emergence of Organ Trafficking

Organ transplantation represents one of modern medicine's greatest achievements, transforming and extending the lives of hundreds of thousands of people annually. However, this medical success has created an unintended consequence: a massive disparity between the number of people who need organ transplants and the number of organs available for transplantation. It is estimated that approximately 2 million lives are affected by organ shortage annually worldwide. This shortage creates both legitimate medical challenges and creates perverse incentives for exploitation. When demand vastly exceeds supply, and when financial desperation meets the desperation of patients facing death from organ failure, criminal networks emerge to exploit this gap.

The practice of transplanting organs from trafficked individuals began emerging in documented cases during the 1990s, though it is likely that isolated cases existed earlier without being formally documented. Early reports came primarily from Egypt, India, and Pakistan, where organs were being systematically procured from vulnerable populations including migrant workers, prisoners, slum dwellers, and the economically disadvantaged. The common thread in these early cases was that organs were obtained through coercion, deception regarding the nature and consequences of organ removal, or exploitation of financial desperation.

2. Notable Historical Cases and Patterns

India (1990s-2000s): The Transplant Tourism Model

India emerged in the 1990s as a major centre for organ transplantation, with significant portions of the organs being transplanted coming from trafficked or coerced donors. Extensive studies and investigations revealed a disturbing pattern in which vulnerable populations, particularly agricultural labourers, sex workers, individuals living in slums, and other economically desperate groups, were systematically coerced into selling kidneys and liver segments. Brokers and middlemen operated networks that identified vulnerable individuals, explained the financial compensation available (which was often far below market rates and far below what international patients paid), and facilitated the medical procedures.



International scrutiny and pressure from the international medical community led to the passage of the **Transplantation of Human Organs Act in 1994**, which attempted to regulate organ transplantation and prohibit commercial trading in organs. However, implementation of this legislation has faced significant challenges, including corruption of officials, lack of enforcement resources, and the continued desperation of poor populations willing to sell organs despite legal prohibitions.

Philippines (2000s): Prison-Based Organ Procurement

The Philippines became the site of a particularly disturbing form of organ harvesting; the systematic procurement of organs from death row inmates. Investigations conducted in the mid-2000s revealed that condemned prisoners were being coerced into providing organs under circumstances that raised profound questions about genuine consent. In some cases, prisoners were not fully informed about what organ removal would entail or its long-term health consequences. In other cases, promises were made that sentences would be commuted or reduced in exchange for organ donation, promises that were subsequently never honoured. This practice represented a grotesque abuse of state power and the vulnerability of prisoners who are already stripped of their rights and freedoms.

The discovery of this practice led to significant public outcry within the Philippines and internationally. Subsequent legislative action was taken to explicitly prohibit organ harvesting from death row inmates, and international pressure was brought to bear on the Philippine government. This case highlighted how state actors themselves can become perpetrators of organ trafficking and demonstrated that the problem extends beyond criminal networks to potentially involve governmental institutions.

China (2000s-2015): State-Sponsored Trafficking Allegations Investigations and reports from international human rights organizations have alleged that China conducted large-scale involuntary organ harvesting from executed prisoners, and potentially from detained Uyghurs and religious minorities. The scale of allegations is enormous, with some reports suggesting that tens of thousands of organs may have been harvested without proper consent. While obtaining independent verification of these allegations has proven extremely difficult due to limited access to Chinese medical records and lack of transparency, the consistency of multiple credible reporting sources lends serious weight to these allegations.



The Transplantation Society and the World Health Organization both expressed serious concerns about the lack of transparency and apparent absence of genuine informed consent in China's organ procurement system. The allegations involve not merely organ trafficking in the traditional sense, but state-sponsored organ harvesting that appears to target vulnerable populations including prisoners, religious minorities, and ethnic minorities. Legislative reforms were subsequently announced by the Chinese government, and officials stated that harvesting organs from executed prisoners would be discontinued and that organ procurement would transition to voluntary donation based on genuine consent. However, questions about the true implementation and enforcement of these stated reforms persist.

Egypt (2000s): Biomedical Exploitation and Medical Tourism Egypt became a significant hotspot for what is known as 'transplant tourism,' where wealthy patients, predominantly from Gulf Arab states, travelled to Egypt to receive organ transplants. These wealthy patients paid substantial sums of money for transplants, often in the range of tens of thousands of dollars per transplant. The donor population consisted primarily of desperately poor Egyptian nationals, migrants working in Egypt, and other economically vulnerable individuals. Most shocking was that these vulnerable donors received only minimal compensation relative to the actual value of the organs and the fees paid by recipients and international brokers.

A comprehensive study conducted in 2009 found that approximately 85% of living kidney donors in Egypt who participated in commercial transplantation arrangements were not adequately informed about the medical risks or long-term health consequences of donating a kidney. Many donors received no pre-operative medical screening and no post-operative follow-up care despite the fact that donating a kidney significantly increases the risk of developing kidney disease later in life. This case exemplifies how transplant tourism creates demand for organs from desperate and vulnerable populations, with little regard for their long-term health and wellbeing.



Mechanisms of Organ Trafficking:

Understanding the mechanisms through which organ trafficking occurs is essential for developing effective prevention strategies. Traffickers and those who facilitate organ trafficking employ several distinct tactics to procure organs from vulnerable populations. These tactics are often used in combination to maximize the likelihood of success and to create psychological pressure on potential donors.

Deception represents one of the most common mechanisms. In deception-based trafficking schemes, victims are promised money, jobs, or other benefits but discover too late, if at all, that organ removal is the actual purpose of the exploitation. Victims may be told they are undergoing routine medical procedures or that they are selling blood or bone marrow, not vital organs. Some victims are never explicitly told that an organ has been removed and only discover this through the physical symptoms they experience afterward. Deception is particularly effective because it bypasses the need for physical coercion or obvious threats, making these trafficking schemes harder to detect.

Coercion represents another critical mechanism. In coercion-based schemes, traffickers and their accomplices use various forms of pressure to extract what appears on paper to be 'consent' but is fundamentally coerced. Coercion can take many forms: debt bondage, in which victims are told they owe money to traffickers and must sell organs to pay off this debt; threats to family members, in which victims are threatened with harm to their relatives if they do not comply; manipulation of immigration status, in which undocumented migrants are threatened with deportation if they do not agree to organ removal; or psychological manipulation that creates a sense of powerlessness and inevitability.

Exploitation of vulnerability represents a third critical mechanism. Certain populations are inherently more vulnerable to trafficking because of their socioeconomic circumstances. Extreme poverty, illiteracy, social marginalization, and lack of access to legal protection and justice systems make certain populations susceptible to organ trafficking. When people face starvation, homelessness, or inability to provide for their families, selling an organ for cash may appear to be a rational economic decision despite the significant health risks. The existence of such vulnerable populations creates a persistent supply of potential organ donors for traffickers to exploit.



Abuse of authority represents a fourth critical mechanism. Medical professionals, border officials, law enforcement officers, and other government officials may participate in organ trafficking networks. When such officials abuse their authority and positions of trust, they can facilitate trafficking with minimal risk of prosecution. A corrupt doctor may inform a vulnerable patient that organ removal is medically necessary when it is not, or may fail to obtain genuine informed consent. A corrupt border official may allow traffickers and trafficking victims to cross borders with impunity. A corrupt law enforcement officer may ignore reports of organ trafficking or may protect traffickers from investigation.



LEGAL AND REGULATORY FRAMEWORK

1. **UN Protocol to Prevent, Suppress and Punish Trafficking in Persons (2000)**

The Palermo Protocol is widely recognized as the primary international instrument addressing human trafficking in all its forms. The Protocol provides a comprehensive definition of trafficking that reads: 'the recruitment, transportation, transfer, harboring or receipt of persons, by means of threat or use of force or other forms of coercion... for the purpose of exploitation.' This definition is broad enough to encompass organ trafficking, as organ trafficking clearly involves obtaining organs through deception or coercion for the purpose of exploiting vulnerable donors. However, the Protocol does not specifically address organ trafficking by name, which creates interpretive challenges for prosecutors who must argue that specific cases of organ procurement constitute human trafficking under the general provisions of the Protocol.

The **Palermo Protocol, 2000** ; established several key principles that guide international efforts to combat human trafficking. The Protocol mandates that all forms of trafficking be criminalized in national legislation, with appropriate criminal penalties. It requires countries to establish victim protection measures and to provide rehabilitation services and assistance to trafficking victims. Critically, the Protocol also establishes mechanisms for international cooperation in investigating trafficking, extraditing traffickers, and sharing evidence across borders. The Protocol has been ratified by the vast majority of countries worldwide and serves as the foundation for international efforts to combat human trafficking generally, including organ trafficking.

2. UN Convention Against Transnational Organized Crime (2000) This Convention directly facilitates international cooperation in combating organized crime, which includes organ trafficking networks that increasingly operate as organized criminal enterprises. Organ trafficking often involves sophisticated, multi-national networks with distinct roles and hierarchies similar to other forms of organized crime. The Convention provides legal tools and frameworks that countries can use to conduct joint investigations of organ trafficking networks, to extradite offenders who flee to other countries, and to share evidence and intelligence across borders. The Convention also provides for protective measures for witnesses and whistleblowers who might otherwise face retaliation from criminal networks.



3. Universal Declaration on Organ Transplantation (1991, Updated 2010)

This Declaration was adopted by the World Health Assembly and establishes fundamental ethical principles that should govern organ procurement and transplantation practices globally. The Declaration establishes several core principles that have become international norms. First, organ procurement must be based on voluntary, informed, and uncompensated donation by donors or their families. While the Declaration acknowledges that minor compensation for expenses incurred in the donation process may be appropriate in some contexts, the fundamental principle is that organs should not be commercialized and that donations should be genuinely voluntary. Second, transplantation procedures must be conducted in ways that respect human dignity and refuse to reduce human beings or their body parts to commodities to be bought and sold. Third, organ procurement processes must be equitable in the sense that access to organs should not be determined solely by ability to pay, and procurement must be transparent so that the public can have confidence in the ethics of transplantation systems.

4. World Health Organization Guiding Principles on Human Cell, Tissue and Organ Transplantation (2010, Revised)

These Guiding Principles were endorsed by WHO member states and provide concrete guidance on implementing ethical transplantation practices. The principles emphasize that each country should aim toward national self-sufficiency in organ procurement, meaning that countries should meet the transplantation needs of their own populations through domestic organ donation rather than relying on organs procured from other countries, particularly lower-income countries. This principle recognizes that transplant tourism and international organ trade create exploitation of vulnerable populations in lower-income countries and place upward pressure on organ procurement practices that may not be ethically sound. The principles also establish an absolute prohibition on commercial organ trading, meaning that organs cannot be bought or sold as commodities. The principles further require transparency in transplantation activities so that the public and regulators can verify that ethical standards are being met. Finally, the principles establish requirements for comprehensive donor protection, including medical screening, informed consent procedures, and post-operative follow-up care.



5. Istanbul Declaration on Organ Trafficking and Transplant Tourism (2008, Revised)

Developed collaboratively by leading transplant professionals, ethicists, and civil society organizations, the Istanbul Declaration specifically condemns transplant tourism and organ trafficking as deeply exploitative practices that cause serious harm to vulnerable populations and undermine trust in transplantation systems. The Declaration called for much stronger oversight and regulation of transplantation systems and outlined specific measures that should be implemented. The Declaration emphasizes the absolute prohibition on buying and selling organs, both to prevent commercialization and to prevent the development of markets for organs that would inevitably exploit vulnerable populations. The Declaration also requires that genuine informed consent be obtained before any organ procurement, and specifies that informed consent means that donors understand the nature of what is being removed, the surgical procedure involved, the short-term and long-term health risks, and any alternatives. Finally, the Declaration calls for the prosecution of traffickers and for the imposition of professional sanctions on transplant professionals who knowingly participate in organ trafficking or transplant tourism.



Questions A Resolution Must Answer (QARMA)

1. How can Member States standardize the legal definitions of TIP for OR and organ commercialism, invalidate coerced consent, and establish extraterritorial jurisdiction to prosecute citizens engaging in cross-border transplant tourism?
2. What binding protocols, independent verification mechanisms, and penal sanctions including license revocations must be enforced against complicit healthcare professionals, diagnostic facilities, and private hospital administrations?
3. How will INTERPOL, national Financial Intelligence Units (FIUs) coordinate to intercept illicit organ transport routes and dismantle criminal financing layered through medical tourism shell companies and cryptocurrency?
4. What international safeguards will ensure absolute legal immunity (non-punishment) for exploited donors, alongside sustainable funding mechanisms for their long-term post-operative healthcare, psychological support, and witness protection?
5. What multilateral strategies should be deployed to curb organ demand through national deceased-donor registries, while protecting refugees, displaced persons, and crisis-affected populations from predatory brokerage networks?